Healthcare Economics & Financing UB Learns Open Book Quiz

1. A 45-year-old man with no significant medical history is concerned about unexpected medical expenses. He asks his physician why people need health insurance. Which of the following best explains the primary financial purpose of health insurance?

A. To ensure equal access to all treatments
B. To reduce the variance of income by protecting against unpredictable costs
C. To guarantee better health outcomes for enrollees
D. To eliminate the risk of illness
Answer: B

Rationale: Health insurance primarily protects individuals from unpredictable, potentially catastrophic medical expenses by pooling risk across many people, thereby reducing financial uncertainty rather than guaranteeing health outcomes or eliminating illness.

2. A 30-year-old woman with private health insurance chooses to see her doctor for multiple minor complaints because her co-pay is only $10 per visit. This is an example of which of the following?

A. Ex ante moral hazard
B. Ex post moral hazard
C. Risk selection
D. Loading cost
Answer: B

Rationale: Ex post moral hazard occurs after obtaining insurance, when lower out-of-pocket costs encourage greater use of healthcare services. The low copay makes the patient more likely to seek care for minor complaints.

3. A 27-year-old man with health insurance decides to start skydiving because he knows that emergency care will be covered if he is injured. This behavior reflects which concept?

A. Ex post moral hazard
B. Adverse selection
C. Ex ante moral hazard
D. Capacity control
Answer: C

Rationale: Ex ante moral hazard refers to engaging in riskier behavior because insurance reduces the financial consequences of illness or injury. Skydiving is an example of increased risk-taking after obtaining coverage.

4. A 65-year-old woman needs surgery costing $20,000. Her insurance policy requires her to pay the first $2,000 before coverage begins. This $2,000 payment is best described as which of the following?

A. Premium
B. Deductible
C. Coinsurance
D. Co-pay
Answer: B

Rationale: A deductible is the amount a patient must pay out of pocket before insurance begins paying for covered services.

5. A 54-year-old man with health insurance is responsible for paying 20% of his $10,000 hospital bill after meeting his deductible. What type of cost-sharing is this?

A. Premium
B. Deductible
C. Co-pay
D. Coinsurance
Answer: D

Rationale: Coinsurance is a percentage of the cost of covered services that the patient pays after meeting the deductible.

6. A 70-year-old woman in Quebec was receiving free prescriptions before a 1996 reform that introduced coinsurance and deductibles. After the reform, she cut back on both essential and non-essential medications and later required hospitalization for heart failure. This demonstrates which of the following unintended consequences of cost-sharing?

A. It improves efficiency by eliminating unnecessary care
B. It may reduce use of essential medications, worsening outcomes
C. It prevents adverse selection in the insurance pool
D. It ensures patients always make rational medication choices
Answer: B

Rationale: Cost-sharing can reduce unnecessary care, but it may also discourage patients from taking high-value medications, leading to poorer health outcomes and potentially more expensive hospitalizations.

7. A 40-year-old man tells his physician that he prefers comprehensive health insurance that covers all costs, even minor services. Which of the following is the first fundamental tradeoff of health insurance this preference highlights?

A. More coverage protects against large risks but increases unnecessary care use
B. Broader coverage reduces risk selection but increases premiums
C. Larger risk pools reduce loading costs but limit patient choice

D. Expanding coverage decreases secondary risk but increases primary risk
Answer: A

Rationale: The first major tradeoff in health insurance is that broader coverage provides better financial protection but also increases the likelihood of moral hazard by reducing the cost of seeking care.

8. A 33-year-old woman in Canada is told she will have to wait 3 months for an elective knee surgery despite having no cost-sharing at the point of service. This waiting time is an example of which of the following strategies to limit moral hazard?

A. Risk corridors
B. Gatekeeping
C. Capacity control
D. Risk adjustment
Answer: C

Rationale: Capacity control limits utilization by restricting the supply of services, such as through waiting lists, rather than charging patients at the point of care.

9. An insurance plan offers free gym memberships, which results in more healthy individuals enrolling compared to sicker individuals. This is best described as which of the following?

A. Adverse selection
B. Favorable risk selection
C. Ex ante moral hazard
D. Ex post moral hazard
Answer: B

Rationale: Favorable risk selection occurs when healthier individuals are disproportionately attracted to a health plan, lowering average healthcare costs for the insurer.

10. A 56-year-old man pays $500 per month for his health insurance. His expected healthcare costs for the year are $6,000. The portion of his premium that equals his expected claims is referred to as which of the following?

A. Loading cost
B. Actuarially fair premium
C. Risk premium
D. Stop-loss
Answer: B

Rationale: The actuarially fair premium equals the expected healthcare costs of the insured before administrative expenses, profit, and other loading costs are added.

11. A 29-year-old patient with HIV buys an ACA marketplace plan. The insurer places all HIV medications, including generics, in the highest cost-sharing tier. This practice is best described as:

A. Adverse selection
B. Risk adjustment
C. Adverse tiering
D. Reinsurance
Answer: C

Rationale: Adverse tiering places expensive medications used by patients with specific conditions into the highest cost-sharing tiers, discouraging those patients from enrolling or remaining in the plan.

12. A 47-year-old woman with diabetes chooses an ACA exchange plan. To protect insurers from very high costs for patients like her, the ACA created a mechanism that pays insurers when they cover unusually expensive individuals. This is known as:

A. Risk corridors
B. Reinsurance
C. Risk adjustment
D. Capacity control
Answer: B

Rationale: Reinsurance protects insurers against exceptionally high-cost enrollees by reimbursing part of those expenses, reducing financial risk.

13. A 34-year-old uninsured man applies for coverage through an ACA marketplace. He learns that plans cannot deny him coverage for pre-existing conditions, and that his premiums may be reduced with tax credits. These provisions were designed primarily to address which of the following?

A. Loading costs
B. Moral hazard
C. Adverse selection
D. Secondary risk
Answer: C

Rationale: ACA provisions such as guaranteed issue and premium tax credits were designed to reduce adverse selection by encouraging both healthy and sick individuals to participate in the insurance market.

14. A 59-year-old man joins a large employer-sponsored health plan with thousands of employees. Compared to buying an individual plan, this large group insurance will primarily reduce which of the following?

A. Primary risk
B. Secondary risk
C. Adverse selection
D. Loading costs
Answer: B

Rationale: Large employer groups reduce secondary (financial) risk by spreading unpredictable healthcare costs across many individuals, making expenses more predictable.

15. A 42-year-old woman compares premiums across multiple ACA exchange plans. Some plans lose money if they attract many high-cost enrollees. To protect against extreme financial losses and gains, the ACA introduced which of the following mechanisms?

A. Risk corridors
B. Reinsurance
C. Stop-loss provisions
D. Gatekeeping
Answer: A

Rationale: Risk corridors were designed to limit extreme insurer gains and losses during the early years of the ACA marketplaces by sharing financial risk with the government.

16. Which is a key advantage of group employer-based insurance in reducing adverse selection?
A. It requires risk-adjusted premiums
B. All employees are automatically covered
C. It excludes high-risk individuals
D. It offers universal public coverage
Answer: B
Rationale: Automatic enrollment through employer-sponsored coverage broadens the risk pool by including healthier individuals who might otherwise remain uninsured, reducing adverse selection.

17. A disadvantage of employer-based insurance is:
A. It encourages overuse of preventive care
B. It restricts employee mobility between jobs
C. It increases insurer competition
D. It reduces adverse selection
Answer: B
Rationale: Employer-sponsored insurance can create 'job lock,' where employees are reluctant to change jobs because they would lose their health insurance.

18. Compared with an HMO, a PPO typically:
A. Requires referrals for specialists
B. Offers no out-of-network coverage
C. Costs more but allows out-of-network care
D. Is more restrictive in provider choice
Answer: C

Rationale: PPOs generally have higher premiums than HMOs but provide greater flexibility by allowing out-of-network care without requiring referrals.

19. Which best describes an EPO?
A. Requires a referral for all specialist visits
B. Allows out-of-network coverage at higher costs
C. Requires use of in-network providers but no referrals
D. Offers the most flexible provider choice
Answer: C
Rationale: EPOs require members to use in-network providers (except emergencies) but typically do not require referrals for specialty care.

20. A point-of-service (POS) plan is best described as:
A. A high-deductible plan with HSA
B. A hybrid of HMO and PPO
C. An indemnity insurance product
D. A catastrophic-only insurance
Answer: B
Rationale: POS plans combine features of HMOs and PPOs by encouraging coordinated in-network care while allowing out-of-network services at higher cost.